

ST. JUDE MEDICAL CENTER - DEPARTMENT OF NEUROLOGY & MEMORY DISORDERS

1200 Healthcare Parkway, Building B, Suite 400 • Confidential Medical Record

Patient Name: Thorne, Marcus Aurelius	Sex: Male	MRN: 6109-2239
DOB / Age: 07/14/1958 (Age: 68)	Exam Date: 08/24/2026	Severity: CRITICAL / Severe
Attending Physician: Dr. Sarah Jenkins, MD (Neurology)	Referring Physician: Forensic Psychiatry Liaison & Adult Protective Services	Status: Completed

REASON FOR EVALUATION

Urgent behavioral neurology evaluation for profound social disinhibition, compulsive criminal theft, financial devastation, severe hyperorality, and volatile aggression. Accompanying family member: Cynthia Thorne (Wife/Legal Guardian).

CLINICAL HISTORY (History of Present Illness)

The patient is a 68-year-old former corporate attorney presenting with a catastrophic 3-year history of personality transformation and frontal cognitive collapse. Family reports severe social disinhibition including public disrobing, unsolicited offensive sexual commentary to strangers, and compulsive shoplifting. Patient recently liquidated family retirement accounts and transferred \$280,000 to offshore wire fraud schemes due to total lack of judgment and anosognosia. He exhibits profound hyperorality—stuffing fists full of raw granulated sugar and attempting to consume plastic cutlery and paper napkins. He demonstrates profound apathy toward family distress, showing zero empathy when informed of a close relative's death, alternating with explosive physical agitation when restricted from pacing or sweets. Functional Assessment: Incapable of managing any instrumental activities of daily living (medications, finances, driving, meal prep, navigation). Banned from driving after multiple collisions. Basic ADLs require constant verbal redirection due to motor impersistence and apathy. Behavioral Symptoms: Extreme disinhibition, severe apathy/loss of empathy, severe hyperorality/dietary changes, compulsive pacing rituals, volatile verbal and physical aggression, severe social alienation.

PAST MEDICAL HISTORY & MEDICATIONS

Past Medical History: Behavioral changes previously misdiagnosed as late-onset bipolar mania; no prior cardiovascular disease. Active Medications: Sertraline 100 mg PO daily (ineffective), Trazodone 50 mg PO at bedtime

NEUROLOGICAL & PHYSICAL EXAMINATION

General: Hyperactive, restless male pacing clinic room, grabbing objects off clinician desk. Complete lack of insight (anosognosia). Cranial Nerves: II-XII intact. Saccadic pursuit jerky. Motor: Utilization behavior present (compulsively drinking from doctor's water cup); motor impersistence (cannot sustain tongue protrusion >3 seconds). Primitive reflexes: Strongly positive grasp reflex and visual rooting reflex. Gait: Wandering pacing gait, rapid but physically intact.

STANDARDIZED COGNITIVE TESTING: Frontal Assessment Battery (FAB) & MMSE (Score: 11/30 - Severe Frontal-Executive Dementia (bvFTD Profile))

- Orientation: 6/10 (Disoriented to day, month, date, institution)
- Registration: 3/3
- Attention Calculation: 0/5 (Complete distractibility, cannot engage in serial 7s)
- Recall: 2/3 (Episodic memory relatively spared compared to profound frontal collapse)
- Language Visuospatial: 0/9 (Severe dynamic aphasia, total echopraxia, concrete perseveration, FAB score 3/18)

DIAGNOSTIC WORKUP & BIOMARKERS

- Laboratory Findings: Comprehensive tox screen negative, heavy metals normal, CSF 14-3-3 protein negative, tau/Abeta42 ratio non-AD pattern.
- Brain MRI Biomarkers: Profound symmetrical 'knife-edge' lobar atrophy of the bilateral frontal lobes and anterior temporal poles. Preserved posterior parietal and minimal hippocampal volume loss (MTA Score: Grade 1). Fazekas Grade 0 (no vascular white matter lesions).

DIAGNOSTIC IMPRESSION

Primary Diagnosis: Major Neurocognitive Disorder due to Probable Behavioral Variant Frontotemporal Dementia (bvFTD), Late Stage (ICD-10: G31.09 / F02.818) Clinical Summary: 68-year-old male with severe behavioral variant frontotemporal dementia featuring catastrophic disinhibition, hyperorality, financial exploitation (\$280k lost), utilization behavior, FAB 3/18, MMSE 11/30, and bilateral knife-edge frontal lobar atrophy, requiring immediate locked psychiatric memory facility placement.

PLAN & CLINICAL RECOMMENDATIONS

- Immediate emergency placement in specialized locked neurobehavioral memory care unit.
- Initiate Quetiapine 25 mg PO BID and titrate for explosive behavioral aggression and pacing.
- Cross-titrate Sertraline to Paroxetine 20 mg PO daily for compulsive behaviors and hyperorality.
- Strict environment safety: lock all food pantries, sharp objects, and toxic household substances to prevent ingestion.
- Finalize emergency legal conservatorship and complete financial power of attorney revocation of accounts.

Electronically Signed: Dr. Sarah Jenkins, MD (Neurology)

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